



NUTRITION, ILLNESS & INFECTION PREVENTION POLICY MANUAL

Nutrition and Menu Planning (Canada's Food Guide · Texture Modification · Allergy Management) · Illness, Injury and Medical Emergency Response and Notification · Infection Prevention, Hand Hygiene, Exclusion, Cleaning and Outbreak Management

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Effective Date	July 1, 2026
Review Date	July 1, 2027 (annual); also upon any funder or public health directive, serious illness/injury incident, outbreak, or BC regulatory change
Applies To	All Evergreen Community Care residential care programs and 1–2 bed homes serving: children and youth in MCFD care; adults receiving CLBC-funded residential services. All Youth Care Workers / Community Support Workers, Shift Leads, Program Coordinators, Program Managers, and the Executive Director.
Funder Streams	M = MCFD (children and youth) · C = CLBC (adults) · Both = both streams. Where a requirement is stream-specific it is marked (M) or (C).
Legislative Authority	Community Care and Assisted Living Act and Residential Care Regulation (where a facility is licensed — 3+ persons in care); Public Health Act and Reporting Information Affecting Public Health Regulation; Food Premises Regulation and FOODSAFE program; Occupational Health and Safety Regulation (WorkSafeBC — exposure control plans); Canada's Food Guide (2019); BC Centre for Disease Control (BCCDC) guidelines; HealthLink BC; Child, Family and Community Service Act (CFCSA) reporting duties (M)
Funder Standards	MCFD Standards for Staffed Children's Residential Services (SHSS):

	Standard D (health and wellbeing, incl. nutrition); Standard C (personal safety); reportable circumstances and critical injury/illness reporting requirements. CLBC contract terms: critical incident reporting, health and safety provisions per the executed service contract.
Related Documents	Medication Administration Manual (ECC-MED-2026-001) — EpiPen/anaphylaxis §23, adverse reactions §36, emergencies §37; Health Care & Consent Policy Manual (ECC-HC-2026-001) — consent frameworks, prescriber access, immunization §9; OHS Program Binder (ECC-OHS-POL-2026-002); RC Program Manual (ECC-RC-POL-2026-003); 1-2 Bed Home Opening & Compliance Binder (ECC-12BED-2026-001)

 This manual governs three linked domains of daily health protection: **what people eat** (nutrition, menus, texture and allergy safety), **what happens when someone is ill or injured** (recognition, response, and who must be notified — MCFD social worker and licensing (M) / CLBC (C)), and **how infection is prevented and outbreaks are managed** (hand hygiene, exclusion, cleaning, and public health notification). Every staff member is trained on this manual before unsupervised duties. When in doubt about any illness or injury: call 811 (HealthLink BC) for clinical advice, 911 for emergencies, and notify the PC the same shift — never wait to see if it gets better on its own.

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VERSION HISTORY

PART 1 — Policy Foundation

1. POLICY STATEMENT AND GOVERNING PHILOSOPHY

Good food, fast recognition of illness and injury, and disciplined infection prevention are the three most ordinary — and most important — ways a residential program protects the health of the people who live there. None of them is optional, and none of them is left to individual habit. This manual sets the agency standard for each.

Principle	What It Means in Practice
Food is care, not catering	Menus follow Canada's Food Guide, reflect the preferences and cultures of the people who live in the home, and are planned — not improvised. Every person eats safely: allergies, texture needs, and medical diets are documented, posted for staff, and followed at every meal and snack without exception.
The person shapes their own food life	People in care help plan menus, shop, and cook to their ability and interest. Food is never used as a reward or punishment, never withheld as a consequence, and portions are never restricted for behavioural reasons. Capable adults (C) make their own food choices; staff support informed choices rather than policing them.
Recognize early, respond fast	Staff monitor wellness every shift and act on change the same shift. The escalation ladder is always available: observe and document → 811 / prescriber → urgent care → 911. Choosing a rung too high is never criticized; choosing one too low is a training issue the PM addresses.
Notification is part of the response	A serious illness or injury is not fully handled until the right people know: the PC/PM, the MCFD social worker and — where the facility is licensed — the licensing officer (M), or CLBC per the critical incident terms of the contract (C). Notification timelines in Section 20 are hard requirements.
Clean hands, clean home	Hand hygiene is the single most effective infection control measure and is a job requirement, not a suggestion. Cleaning follows the posted schedule (Appendix E) and is documented. Ill staff stay home; ill residents are supported and, where indicated, excluded from group activities per Appendix G.
Outbreaks are managed, not weathered	Two or more linked cases of GI or respiratory illness triggers the outbreak protocol (Section 29): enhanced cleaning, the line list, exclusion, and a call to public health. Early over-reaction is cheaper than late under-reaction.
Dignity throughout	Illness, feeding support, texture modification, and infection control measures are delivered with privacy

Principle	What It Means in Practice
	and respect. Infection precautions never become social isolation beyond what health protection actually requires.

2. LEGISLATIVE AND REGULATORY FRAMEWORK

Authority	Stream	Relevance and Key Requirements
Canada's Food Guide (2019)	Both	The national nutrition standard adopted by this manual: plenty of vegetables and fruits; whole grain foods; protein foods (with plant-based protein regularly); water as the drink of choice; limiting highly processed foods high in sodium, sugars, and saturated fat. The Guide also emphasizes food skills, cooking together, and eating with others — all program practices.
MCFD SHSS Standard D	M	Youth in care receive nutritious food meeting their developmental, cultural, and health needs; health and wellbeing needs are monitored and met; serious illness, injury, and other reportable circumstances are reported per MCFD requirements.
MCFD reportable circumstances / critical injury and illness reporting	M	Prescribed incidents — including serious injury, serious illness requiring medical attention or hospitalization, and any incident affecting a child's safety — are reported to the MCFD social worker (and after hours to Centralized Screening 1-800-663-9122) within the required timelines, with written incident reports to follow.
CLBC service contract — critical incident reporting	C	Critical incidents — including serious injury, unexpected serious illness, hospitalization, and outbreaks affecting service delivery — are reported to CLBC per the executed contract (verbal notification promptly, written CI report within the contract timeline, commonly 24 hours). The PM confirms the exact timelines in the contract and posts them at the program.

Authority	Stream	Relevance and Key Requirements
Community Care and Assisted Living Act / Residential Care Regulation	Both (licensed facilities only)	Where a facility is licensed (3+ persons in care), the RCR imposes nutrition, reportable incident, and infection control duties, and reportable incidents are also reported to the Licensing Officer. A 1–2 bed home is not licensed — the funder contract and this manual are the compliance framework — but this manual adopts licensing-level practice as the agency standard everywhere.
Public Health Act and regulations	Both	Duty to cooperate with medical health officers; certain communicable diseases are reportable by health professionals; programs report suspected outbreaks to the local public health unit and follow MHO directions.
Food Premises Regulation / FOODSAFE	Both	Safe food handling standards. Agency standard: at least one person per shift responsible for food preparation holds FOODSAFE Level 1 (or equivalent); safe cooking, cooling, and storage temperatures are followed and fridge/freezer temperatures logged.
WorkSafeBC OHS Regulation	Both	Exposure control plan for blood and body fluids; PPE provision and training; first aid requirements for the workplace (see OHS Program Binder ECC-OHS-POL-2026-002).
BCCDC and HealthLink BC (811)	Both	Provincial clinical guidance for communicable disease, exclusion, and outbreak management; 811 provides 24/7 nursing advice and is the default clinical consult line for illness that is concerning but not clearly an emergency.
CFCSA s.14 (M)	M	Any reason to believe a child needs protection — including injury patterns inconsistent with explanation — must be reported to a child welfare worker. This duty overrides all internal procedure.

3. ROLES AND RESPONSIBILITIES

Role	Responsibilities Under This Manual
Youth Care Worker / Community Support Worker	Prepare and serve meals per the posted menu and each person's Special Diet, Texture and Allergy Plan (Appendix B); check the allergy board before preparing any food; complete wellness monitoring every shift and document; respond to illness/injury per Part 3 and report to the PC same shift; practice hand hygiene per Section 23 and complete assigned cleaning per Appendix E; stay home when ill per Appendix G; complete the Illness/Injury Report (Appendix C) before end of shift for any reportable event.
Shift Lead	All worker responsibilities plus: verify at shift start that the allergy board, menu, and any current illness precautions are posted and current; lead the immediate response to any medical event on shift; initiate the notification chain (Section 20); ensure exclusion decisions for that shift follow Appendix G.
Program Coordinator (PC)	Build and post the 4-week menu with resident participation; maintain Appendix B plans for every person and reconcile them with the Individual Medication Profile allergies (ECC-MED §7) and Health Care Profile (ECC-HC App. A); maintain food safety logs (fridge/freezer temperatures, FOODSAFE currency); receive all illness/injury reports and complete PC-level notifications; maintain the cleaning schedule and audit completion; recognize potential outbreaks and initiate the outbreak protocol with the PM; track staff illness exclusions.
Program Manager (PM)	Approve menus quarterly; approve every Appendix B plan; make or confirm all funder notifications for serious illness/injury (Section 20) and outbreaks (Section 30); liaise with licensing (licensed facilities), public health, and the funder; review every Appendix C report within 24 hours and every emergency within 5 business days; ensure training currency (FOODSAFE, first aid, this manual).
Executive Director (ED)	Approve this manual annually; receive quarterly trend reports (illness/injury, outbreaks, food safety audits); ensure resourcing (food budget adequate for Food Guide-quality menus, PPE and cleaning supplies always in stock); respond to any licensing or public health directive.
Prescriber / Dietitian / Speech-Language Pathologist	Order medical diets, texture modifications, and swallowing assessments; the program implements exactly as ordered and never modifies texture levels without clinical direction.

Role	Responsibilities Under This Manual
Public Health Unit / Medical Health Officer	Directs outbreak management once notified; the program follows MHO direction and documents it.

4. KEY DEFINITIONS

Term	Definition as Used in This Manual
Medical diet	A diet ordered by a prescriber or dietitian for a health condition (e.g., diabetic, renal, low-sodium, celiac/gluten-free, ketogenic). Implemented exactly as ordered; changes only on clinical direction.
Texture-modified diet	Food and/or fluid consistency altered for swallowing safety per clinical direction, described using the IDDSI framework (levels 0–7: thin to regular; minced and moist; pureed; thickened fluids).
Anaphylaxis	A severe, rapidly progressing allergic reaction that can be fatal. Managed per ECC-MED §23: EpiPen immediately, 911 immediately.
Serious illness	An illness that requires emergency care, hospital assessment or admission, or that a prescriber identifies as serious; or any illness with rapid deterioration, altered consciousness, breathing difficulty, uncontrolled pain, or dehydration risk.
Serious injury	An injury requiring medical treatment beyond program-level first aid — including suspected fractures, head injury with any red flag, burns beyond minor first-degree, wounds requiring closure, or any injury requiring hospital assessment.
Reportable event (M)	Any circumstance on the MCFD reportable circumstances list, including serious illness/injury, hospitalization, and medication errors per ECC-MED. Reported to the social worker (after hours: 1-800-663-9122) per Section 20.
Critical incident (C)	Any event defined as a critical incident in the CLBC contract, including serious injury, unexpected serious illness, and hospitalization. Reported per Section 20.
Routine practices	The baseline infection prevention used with every person, every time: hand hygiene, risk assessment, PPE as indicated, respiratory etiquette, environmental cleaning, safe handling of linen/waste/sharps.
Exclusion	Temporary restriction of an ill person from group settings (school, work, day program, communal meals as applicable) or an ill staff member from work, per Appendix G criteria and public health guidance.
Outbreak	For program purposes: 2 or more persons (residents or

Term	Definition as Used in This Manual
	staff) with similar GI or respiratory symptoms linked in time and place, or any single case of a serious reportable communicable disease. Triggers Section 29.
Line list	The dated record of every symptomatic person during an outbreak: onset, symptoms, status, exclusion dates (Appendix F). It is what public health will ask for first.

PART 2 — Nutrition and Menu Policy

5. NUTRITION STANDARDS — CANADA'S FOOD GUIDE

All menus and food provided by Evergreen Community Care follow Canada's Food Guide (2019), adjusted for each person's age, health conditions, activity, culture, and clinical orders.

Standard	Program Practice
Plate composition	Meals are planned around the Food Guide plate: about half vegetables and fruits; a quarter whole grain foods; a quarter protein foods, with plant-based proteins (beans, lentils, tofu, nuts where allergy-safe) served regularly alongside meat, fish, eggs, and dairy.
Water first	Water is the default drink, freely available at all times. Milk / fortified alternatives are offered at meals. Sugary drinks (pop, fruit drinks, sports/energy drinks) are not purchased on the program grocery budget; juice, if served, is 100% juice in small portions.
Limit highly processed foods	Deep-fried foods, processed meats, packaged snacks high in sodium/sugar, and desserts are occasional foods — not daily menu items. Home cooking from whole ingredients is the norm.
Meals and snacks	Three meals and at least two planned snacks daily; a person who misses a meal (school, work, appointment, sleep) is always offered a full alternative — food is never forfeited by absence.
Growth and appetite (M)	Children and youth receive portions appropriate to age and appetite with seconds available; adolescents' higher energy needs are planned for. Appetite changes are documented (they are also medication-monitoring data — ECC-MED §35).
Adult choice (C)	Capable adults choose what and how much they eat. Staff provide appealing Food Guide-aligned options, information, and support for any goals the adult sets — never restriction, surveillance, or commentary on the adult's body or eating.
No food-based behaviour management	Food, snacks, or preferred foods are never withheld, delayed, or awarded as behaviour consequences.

Standard	Program Practice
	Dessert is not contingent on finishing a meal. This mirrors ECC-BSP prohibited practices.
Special occasions	Birthdays, cultural and religious holidays, and personal celebrations are marked with appropriate foods chosen with the person — celebration food is part of a healthy food culture, not a policy exception to be rationed.

6. MENU PLANNING AND THE 4-WEEK ROTATION

Requirement	Standard
4-week rotating menu	The PC maintains a 4-week rotating menu (Appendix A) covering all meals and snacks, reviewed and refreshed at least quarterly and seasonally. The current week's menu is posted in the kitchen.
Resident participation	Menus are planned with the people who live in the home: regular menu input (house meetings, individual preferences), participation in grocery shopping and cooking to ability and interest, and at least one resident-chosen meal per week per person.
Substitutions	Day-of substitutions are permitted (availability, preference, spontaneity) provided the substitute is of equivalent nutritional quality and respects every Appendix B plan in the home; the substitution is noted on the posted menu.
Special diets integrated	The master menu shows, for each meal, the variation for each person's medical diet, texture level, allergy avoidance, and cultural/religious requirements — one menu, individualized columns; no person's needs are an afterthought.
PM approval	The PM approves the rotation and each quarterly refresh, checking Food Guide alignment, Appendix B compliance, budget adequacy, and variety.
Records	Posted menus (with substitutions noted) are retained for 12 months — they are the record that nutrition standards were met and are reviewable by the funder or licensing.

7. SPECIAL DIETS — MEDICAL, CULTURAL AND PERSONAL

Diet Type	Standard
Medical diets	Implemented exactly as ordered by the prescriber or dietitian and documented on Appendix B with the order attached. Common examples: diabetic meal planning (consistent carbohydrate; coordinate with

Diet Type	Standard
	ECC-MED \$24 glucose monitoring), celiac (strict gluten-free including cross-contamination controls), renal, low-sodium, high-energy. Staff never liberalize or tighten a medical diet on their own judgment.
Cultural and religious diets	Halal, kosher, vegetarian/vegan, fasting practices, and other cultural or religious food requirements are documented on Appendix B at admission, planned into the menu, and followed with the same rigour as medical diets. Questions about observance are answered by the person (and family/community as the person wishes) — not assumed.
Personal preferences and aversions	Strong preferences and aversions are recorded and reasonably accommodated. A person is never forced, pressured, or shamed to eat any food. A refused meal is met with an alternative, not a standoff.
Nutrition support / enteral feeding	Any person with enteral (tube) feeding has a clinician-written feeding plan; only staff trained on that person's plan provide feeds; feeds, tolerance, and site care are documented per the plan. Enteral feeding is a delegated/clinical task coordinated with the health authority — the PM confirms training and authorization before any staff member performs it.
Disordered eating concerns	If staff observe possible disordered eating (persistent restriction, bingeing, purging, rapid weight change, food hoarding driven by distress), they document objectively, notify the PC, and the PC engages the prescriber. Staff do not impose diets, weigh the person, count calories, or set food rules on their own initiative — clinical direction governs, and the tone at the table stays neutral and supportive.

8. TEXTURE MODIFICATION AND SWALLOWING SAFETY (IDDSI)

Texture-modified diets exist to prevent choking and aspiration in people with dysphagia. They are clinical orders, not kitchen preferences.

Requirement	Standard
Clinical assessment and order	Texture modification is implemented only on the direction of a speech-language pathologist, occupational therapist, dietitian, or prescriber. The order — expressed using the IDDSI framework (e.g., Level 4 Pureed; Level 5 Minced & Moist; Level 6 Soft & Bite-Sized; fluids Level 0–3) — is attached to Appendix B.
Every food, every fluid, every time	The prescribed texture applies to all meals, snacks, and drinks, including outings, takeout, and celebration foods. Staff check Appendix B before serving anything

Requirement	Standard
	to a person with a texture order.
Preparation standards	Staff preparing modified textures are oriented to the person's plan and IDDSI descriptors; consistency is verified before serving (e.g., IDDSI fork/spoon tests as trained); thickening agents are used per product instructions and measured — never eyeballed.
Mealtime supervision	The person's plan states required supervision and positioning (upright 90°, remain upright after meals as directed), pacing, cueing, and adaptive equipment. Staff providing feeding assistance are trained on the individual plan and never rush a meal.
Signs of swallowing trouble	Coughing or choking during meals, wet or gurgly voice after swallowing, food pocketing, drooling, recurrent chest infections, or meal refusal in a person with dysphagia are reported to the PC same shift; the PC arranges reassessment. Any suspected aspiration with breathing difficulty: 911 (Part 3).
Never downgrade or upgrade unilaterally	Staff never change a texture level — in either direction — without a documented clinical order. 'They managed regular toast fine yesterday' is an observation to report, not authorization to change the plan.

9. FOOD ALLERGY AND ANAPHYLAXIS MANAGEMENT

 Allergy information lives in three places that must always agree: the Individual Medication Profile (ECC-MED §7), the Individual Health Care Profile (ECC-HC Appendix A), and the Appendix B food plan / kitchen allergy board of this manual. The PC reconciles all three at admission, at every change, and at every annual review. A discrepancy between them is treated as an incident and fixed the same day.

Requirement	Standard
Documentation and posting	Every food allergy and intolerance, the reaction type, and severity is documented on Appendix B. A kitchen allergy board (visible at the point of preparation, respectful in wording) lists each person's allergens; staff check it before preparing or serving any food, including guests' and staff food entering the home.
Allergen control in the kitchen	For anaphylactic allergens: the allergen is not stocked in the home wherever practicable; where it must be present, it is stored segregated and clearly labelled, and preparation uses separate utensils, cutting boards, and cleaning to prevent cross-contact. Label-reading is mandatory for every purchased product — 'may contain' warnings are treated as containing the allergen for anaphylactic persons.

Eating out, school, work and outings	The person's allergy information travels with them; staff planning outings confirm safe options in advance; schools and day programs are informed in writing (with consent per ECC-HC frameworks); the EpiPen travels with the person per ECC-MED §23.
EpiPen readiness	Where any person has a prescribed epinephrine auto-injector: it is stored unlocked, labelled, and accessible per ECC-MED §23.2, checked monthly for expiry, and every staff member can demonstrate use. Anaphylaxis response is ECC-MED §23.1: EpiPen immediately, 911 immediately, never wait to see if it improves.
New or suspected allergies	Any new reaction (hives, swelling, GI symptoms, breathing changes after a food) is treated per Part 3, documented, reported to the PC, and the food is withheld pending prescriber review; Appendix B and the allergy board are updated the same day on a precautionary basis.
Intolerances and sensitivities	Non-allergic intolerances (e.g., lactose) are documented and accommodated in the menu; they are managed for comfort, not with anaphylaxis-level controls, and the distinction is recorded so staff calibrate correctly.

10. CHOKING PREVENTION

- High-risk foods (whole grapes, hot dog rounds, hard candies, popcorn, nuts, chunks of raw hard vegetables, sticky spoonfuls of nut butter) are cut, modified, or avoided for young children (M) and for any person with a swallowing or choking risk identified on Appendix B.
- Meals are eaten seated and unhurried; eating while walking, lying down, or in a moving vehicle is discouraged for everyone and prevented for persons with identified risk.
- Every staff member holds current first aid including choking response for the ages served; response to choking is immediate first aid and 911 per Part 3.
- Any choking event — even fully resolved by first aid — is documented on Appendix C, reported per Section 20 if medical attention was required, and triggers PC review of the person's Appendix B plan.

11. FOOD SAFETY AND FOODSAFE

Requirement	Standard
Training	At least one person on every shift where food is prepared holds FOODSAFE Level 1 (or recognized equivalent); certification currency is tracked in the monthly staff currency check (ECC-12BED §4).
Temperatures	Refrigerators at 4°C or below; freezers at -18°C or below; temperatures logged daily on the posted log. Cook to safe internal temperatures (poultry 74°C+, ground meats 71°C+, reheated leftovers 74°C); use a probe thermometer, not colour.

Requirement	Standard
Danger zone	Perishable food is not held between 4°C and 60°C for more than 2 hours; hot foods held hot, cold foods held cold; leftovers cooled promptly, labelled with date, used within 3 days or frozen; when in doubt, throw it out.
Hygiene in the kitchen	Hand hygiene before and during food preparation (Section 23); no bare-hand contact with ready-to-eat food where gloves/utensils are practicable; separate boards for raw meat; sanitize food-contact surfaces after raw protein; staff with GI illness do not prepare food (Appendix G).
Storage	Food stored covered, labelled, off the floor; chemicals stored away from food (and locked per the facility safety standard); donated or home-preserved foods accepted only when source and safety can be verified.

12. MEALTIME ENVIRONMENT, DIGNITY AND PARTICIPATION

- Meals are shared, social, and unhurried wherever possible — staff sit and eat with the people who live in the home; devices are set aside by agreement of the house rather than imposed rule.
- Food skills are actively taught: menu planning, budgeting, shopping, cooking, and food safety are part of independence development (M: life skills; C: per the adult's goals and plan).
- Feeding assistance, adapted equipment, and texture-modified meals are provided discreetly and presented attractively — pureed meals are plated with the same care as everyone else's.
- A person's eating is never discussed critically in front of them or others; observations belong in the shift log, not table talk.

13. NUTRITION MONITORING AND ESCALATION

Trigger	Response
Meaningful appetite change (increase or decrease) sustained over days	Document in the shift log with specifics; PC reviews against medication profile (many psychotropics affect appetite — ECC-MED §22/§35) and raises with the prescriber.
Unintended weight change observed (clothing fit, visible change)	PC arranges prescriber review; weight is measured only as clinically directed and always privately; no routine program weighing without clinical reason.
Persistent refusal of meals / fluid intake concern	Same-shift PC notification; hydration monitored; prescriber or 811 consulted within 24 hours; dehydration red flags (Section 15) escalate immediately.
Swallowing difficulty signs (Section 8)	Same-shift PC notification; reassessment arranged; interim precautions per the existing plan.
Suspected disordered eating (Section 7)	Objective documentation; PC to prescriber; no program-initiated diets or weighing; supportive neutral

Trigger	Response
	mealtime approach maintained.

PART 3 — Illness, Injury and Medical Emergency Policy

14. DAILY WELLNESS MONITORING

- Staff observe each person's wellness every shift — appetite, fluids, sleep, energy, mood, pain, skin, breathing, elimination where relevant — and document meaningful observations in the shift log. Wellness checks follow the program's daily log / wellness-check format (ECC-12BED §2.5).
- Baseline matters: staff know each person's normal (communication style, activity level, typical complaints) from the Health Care Profile (ECC-HC App. A) so that change is recognizable. For people who do not report symptoms verbally, staff monitor behavioural indicators of pain or illness (changes in eating, sleep, activity, irritability, guarding) with particular care.
- Any concerning change is reported to the PC the same shift and handed over explicitly at shift change — never left implicit in the log.
- Persons in care are supported to understand and report their own symptoms; being believed is program policy — a symptom report is acted on, not screened for credibility.

15. RESPONSE TO ILLNESS

Level	Presentation (examples)	Response
Mild — manage at home	Common cold, mild sore throat, low-grade fever managed with fluids and rest, mild GI upset, minor rash without other symptoms	Comfort care; fluids; rest; fever/pain management only per authorized OTC/PRN protocols (ECC-MED §18-19 — never improvised dosing); document in shift log; apply exclusion criteria (Appendix G); monitor each shift; PC informed same shift.
Moderate — clinical advice same day	Fever persisting beyond 24-48 hours or 38.5°C+ not settling; persistent vomiting or diarrhea; significant pain; wheeze without distress; suspected infection (ear, urinary, skin); any illness in a person with significant chronic conditions	Call 811 (HealthLink BC) or the prescriber's office same day and follow direction; arrange same-day or next-day appointment as advised; increase monitoring frequency; document advice received and actions; PC notified same shift; consent for treatment per ECC-HC frameworks.
Severe — urgent care / emergency	Signs of dehydration (minimal urine, lethargy, sunken eyes); difficulty breathing; chest pain; uncontrolled pain; confusion or unusual drowsiness; fever with stiff neck, severe headache, or non-blanching rash; fever in a person who is immunocompromised	Urgent care or emergency department without delay — or 911 per Section 17 criteria; a staff member accompanies; emergency information sheet travels (ECC-HC Part 5); PC/PM notified per Section 20.

Fever notes: temperature is measured with the program thermometer and documented with time and route. Fever in an infant under 3 months (if ever in care), any fever with a seizure, and fever with the red flags above are emergencies. Antipyretics are given only under the person's authorized OTC/PRN protocol.

16. RESPONSE TO INJURY

Injury	Response
Minor (small cuts, scrapes, minor bruises, minor burns ≤ small first-degree)	First aid per training; document in shift log and on the minor-injury record; monitor; PC informed same shift. Patterns of minor injuries are themselves reportable observations.
Requires medical attention (wounds possibly needing closure, suspected sprain/fracture, burns beyond minor, dental trauma, animal/human bites, eye injuries, any injury with significant pain or reduced function)	First aid; clinical advice (811/prescriber) or direct transport to urgent care/ED as severity indicates; do not splint/relocate beyond first aid training; a staff member accompanies; Appendix C report; notifications per Section 20.
Serious / emergency (suspected spinal injury, major bleeding, compound fracture, serious burns, head injury red flags, unconsciousness)	911; first aid within training (pressure for bleeding; do not move suspected spinal injury except for immediate danger or airway); Section 17–18; notifications per Section 20.
Injury of unknown origin or inconsistent with explanation	Treat the injury per severity above AND: document objectively (photographs per program practice and consent framework); PC and PM notified immediately; (M) child protection duty under CFCSA s.14 applies — report to a child welfare worker; (C) adult protection concerns are reported to the designated agency (Adult Guardianship Act) and CLBC per contract. Internal convenience never delays a protection report.

17. MEDICAL EMERGENCIES — CALL 911 IMMEDIATELY

Call 911 for	While Waiting
Unresponsiveness or altered consciousness; not rousable	Airway, breathing, circulation per first aid training; recovery position if breathing and no spinal concern; do not leave the person
Difficulty breathing; choking not relieved by first aid; blue lips	Choking first aid per training; position of comfort for breathing difficulty; assist with rescue inhaler if prescribed (ECC-MED §16/§20)
Anaphylaxis (throat swelling, hives with breathing difficulty, rapid onset after allergen)	EpiPen immediately per ECC-MED §23.1 — before or simultaneous with the 911 call; second dose after 5–15 minutes if no improvement and available
Suspected overdose or poisoning	Poison Control 1-800-567-8911 simultaneously; do not induce vomiting unless directed; bring containers to hospital (ECC-MED §37)
Seizure lasting over 5 minutes, first-ever seizure, repeated seizures, or seizure with injury/breathing	Do not restrain; clear hazards; protect the head; time the seizure; recovery position after; see Section 18

Call 911 for	While Waiting
difficulty	
Chest pain, signs of stroke (FAST: face droop, arm weakness, speech difficulty — time to call)	Keep the person still and calm; note symptom onset time for paramedics
Major bleeding, major trauma, serious burns, suspected spinal injury	Direct pressure for bleeding; cool (not ice) burns; do not move suspected spinal injury unless in immediate danger
Suicide attempt or serious self-harm	911; stay with the person; remove immediate dangers as safely possible; supportive presence; PM and funder notifications per Section 20 and the program's critical incident procedures
 911 is never delayed for consent, notification, documentation, or to seek permission from a supervisor. Call first. Notify after (ECC-HC Part 5). No staff member will ever face criticism for calling 911 in good faith.	

18. HEAD INJURY, SEIZURE AND OTHER HIGH-RISK PRESENTATIONS

Presentation	Standard
Head injury — red flags	911 / immediate ED for: any loss of consciousness; repeated vomiting; worsening headache; confusion, unusual drowsiness, or behaviour change; unequal pupils; seizure; clear fluid or blood from ears/nose; fall from height or high-mechanism injury; any head injury in a person on anticoagulants (ECC-MED §24).
Head injury — no red flags	811/prescriber advice same day; structured observation for 24–48 hours per advice (waking checks if directed); documented observation schedule in the shift log; explicit handover between shifts; any red flag emerging → 911.
Seizure — known seizure disorder	Follow the person's seizure plan (Appendix to their Health Care Profile): timing, positioning, rescue medication only if prescribed and staff authorized (ECC-MED), post-ictal care, and the plan's own 911 criteria; every seizure is documented (time, duration, description) and reported to the PC; breakthrough seizures reported to the prescriber (ECC-MED §24).
Diabetic emergencies	Hypoglycemia (shaky, sweaty, confused, drowsy): follow the person's documented protocol — fast sugar if safely able to swallow; unresponsive or unable to swallow → 911, do not give oral sugar. Hyperglycemia red flags (vomiting, deep rapid breathing, fruity breath, drowsiness) → urgent clinical contact / 911.
Sepsis awareness	Infection plus any of: very high or very low temperature, rapid breathing, rapid heart rate, confusion, mottled or ashen skin, 'worst they've ever

Presentation	Standard
	looked' → treat as an emergency; say the word 'sepsis' to 911/ED.
Mental health crisis	Imminent risk to self or others: 911 (request mental-health-trained responders where available); a staff member stays with the person, de-escalates per ECC-BSP, and follows the person's crisis plan; Mental Health Act processes are hospital-led (ECC-HC §29); funder notification per Section 20.

19. FIRST AID READINESS

- Every staff member holds a current first aid certificate appropriate to the ages served before working unsupervised (ECC-12BED §2.4); currency is tracked monthly.
- A stocked first aid kit is maintained at the home and in every program vehicle; contents checked monthly on the facility checklist; used items replaced immediately.
- Emergency numbers (911, Poison Control 1-800-567-8911, 811, on-call supervisor, program prescriber, MCFD after-hours 1-800-663-9122) are posted beside every phone, alongside the Appendix D quick reference.
- First aid provided is always documented on Appendix C (or the minor-injury record for minor events), including what was done, by whom, and the person's response.

20. NOTIFICATION REQUIREMENTS — SERIOUS ILLNESS AND INJURY

Notification is a core part of the emergency response, not an administrative afterthought. The Shift Lead starts the chain; the PC and PM complete it. All times and names are documented on Appendix C.

Event	Internal	Funder / External — MCFD stream (M)	Funder / External — CLBC stream (C)
Serious illness (emergency care, ED assessment, or hospitalization)	PC immediately; PC → PM within 30 minutes	MCFD social worker same day (after hours: Centralized Screening 1-800-663-9122); written reportable circumstance report per MCFD requirements (within 24 hours); where the facility is licensed: Licensing Officer notified per RCR reportable incident requirements	CLBC verbal notification promptly (same day) and written critical incident report per contract (commonly within 24 hours); PM confirms and posts the contract's exact timelines
Serious injury (medical treatment beyond first aid)	Same as above	Same as above; injuries of unknown origin additionally engage CFCSA s.14 reporting	Same as above; suspected abuse/neglect additionally engages Adult Guardianship Act designated agency
Hospitalization (any admission)	PC and PM same shift	Social worker same day; medication and records	CLBC per contract; substitute decision-maker/family contacts

Event	Internal	Funder / External — MCFD stream (M)	Funder / External — CLBC stream (C)
		procedures per ECC-MED §34 / ECC-HC §10	per ECC-HC Part 4/5
Death of a person in care	PM and ED immediately	911/police per circumstances; MCFD immediately; licensing immediately (licensed facilities); coroner per law; medications secured per ECC-MED §34	911/police per circumstances; CLBC immediately per contract; coroner per law; PGT/committee/representative and family contacts
Choking requiring intervention, anaphylaxis, overdose, seizure requiring 911	PC immediately; PM within 30 minutes	Reportable per MCFD requirements same day (medication-related events also per ECC-MED §29–30)	Critical incident per contract (medication-related events also per ECC-MED §29–30)
Communicable disease outbreak (Section 29)	PC → PM same day	Social worker informed; licensing informed (licensed facilities); public health per Section 30	CLBC informed per contract; public health per Section 30

 The PM personally confirms that every funder notification required by this table was made and documented — 'I assumed the PC called' is a finding, not an explanation. For the CLBC stream, the executed contract's critical incident definitions and timelines govern where they are stricter than this table; the PM posts them at the program.

21. DOCUMENTATION, FOLLOW-UP AND REVIEW

- Every reportable illness, injury, or emergency is documented on the Illness/Injury Report and Notification Checklist (Appendix C) before the end of the shift, in objective language (what was observed, what was done, times, who was notified).
- Follow-up care (discharge instructions, new prescriptions per ECC-MED §8, follow-up appointments per ECC-HC Appendix E, activity restrictions) is entered into the care record and handed over explicitly until completed.
- The PM reviews every Appendix C report within 24 hours and every emergency within 5 business days (jointly with the ECC-HC §22 review where hospital care occurred): was recognition timely, was the response right, were notifications complete, and what — if anything — should change. Each review with findings generates a corrective action with a named owner (mirroring ECC-MED §31).
- Illness and injury data are trended quarterly to the ED: frequency, type, location, time of day, and any pattern by person (which may indicate an unmet health, supervision, or environmental need).

PART 4 — Infection Prevention and Outbreak Policy

22. ROUTINE PRACTICES — THE BASELINE

Routine practices are used with every person, every time, regardless of diagnosis — because infectious status is often unknown. They are: point-of-care risk assessment (what am I about to do, what could I contact?), hand hygiene, PPE as indicated by the task, respiratory etiquette, environmental cleaning, and safe handling of linen, waste, and sharps.

23. HAND HYGIENE

Requirement	Standard
When — the essential moments	Before food preparation, serving, and eating; before and after any care contact or first aid; before medication administration (ECC-MED §13); after washroom use or assisting with toileting; after handling body fluids, soiled laundry, or waste; after coughing/sneezing/blowing nose; after removing gloves; after animal contact; on arriving at and leaving work.
How	Soap and warm water for at least 20 seconds (mandatory when hands are visibly soiled and after washroom/body fluid contact); 70%+ alcohol-based hand rub acceptable otherwise. Technique per first aid / FOODSAFE training: all surfaces, between fingers, thumbs, nails.
Enablement	Every washroom and the kitchen has soap and single-use towels (or clean cloth towels changed daily); alcohol rub available at the medication area and entrances (stored away from young children (M)); supplies are on the standing order list — never allowed to run out.
People in care	Hand hygiene is taught and supported at teachable moments (before meals, after washroom) at each person's level of ability, with assistance provided as needed and dignity preserved.
Respiratory etiquette	Cough or sneeze into the elbow or a tissue; tissue directly into waste; hand hygiene after. Masks are offered to a symptomatic person for shared spaces where tolerated (never forced), and worn by staff per Section 24.

24. PERSONAL PROTECTIVE EQUIPMENT (PPE)

PPE	When	Notes
Gloves	Anticipated contact with blood/body fluids, mucous membranes, non-intact skin; handling soiled laundry/waste; cleaning with disinfectants; topical medications (ECC-MED §15)	Single use; hand hygiene after removal — gloves are never a substitute for hand hygiene

PPE	When	Notes
Mask (medical)	Providing close care to a person with respiratory symptoms; when directed by public health during outbreaks or respiratory season; when a staff member is recovering per Appendix G return conditions	Fit over nose and mouth; change when moist; hand hygiene after touching/removing
Eye protection	Any risk of splash/spray to the face (vomiting, coughing during close care, cleaning up body fluids)	Reusable eye protection is cleaned and disinfected after use
Gown / apron	Anticipated soiling of clothing (significant body fluid cleanup, care of a person with vomiting/diarrhea during outbreak precautions)	Disposable preferred; remove before leaving the care area; hand hygiene after

PPE is stocked at the home, its location known to all staff, and supply checked on the monthly facility checklist. Donning/doffing is covered in orientation and refreshed annually; the exposure control plan in the OHS Program Binder (ECC-OHS-POL-2026-002) governs occupational exposure.

25. ILLNESS EXCLUSION — PERSONS IN CARE AND STAFF

Exclusion prevents one case from becoming an outbreak. Appendix G is the posted quick reference; the standards below govern.

Who	Standard
Person in care — GI illness	Vomiting or diarrhea: excluded from school, work, day program, and communal food preparation until at least 48 hours symptom-free; supported at home with hydration and monitoring (Section 15); dedicated washroom where the home layout allows during symptoms.
Person in care — respiratory illness	Fever and/or significant respiratory symptoms: stays home from school/work/day program until fever-free 24 hours without antipyretics and symptoms improving; participates in home life as tolerated with respiratory etiquette; per public health direction for specific diseases.
Person in care — specific conditions	Communicable conditions with defined exclusion periods (e.g., strep throat until 24 hours of antibiotics; conjunctivitis per prescriber; chickenpox until lesions crusted) follow prescriber and public health direction, documented on Appendix C/G.
Staff — mandatory self-exclusion	Staff with vomiting or diarrhea do not work until 48 hours symptom-free — no exceptions, including 'just food handling avoidance'. Staff with fever or influenza-like illness do not work until fever-free 24 hours and improving. Staff with infected skin lesions on hands cover them fully and do not prepare food; weeping

Who	Standard
	lesions that cannot be covered: no work until healed or cleared.
Staffing pressure never overrides exclusion	The PM maintains the emergency staffing plan (ECC-12BED §2.4) precisely so that exclusion decisions are never bent to fill a shift. A staff member who works ill 'to help out' has created a larger problem than an open shift.
Visitors	Visitors with GI or febrile respiratory illness are asked to defer their visit; during declared outbreaks, visiting follows public health direction, balanced against the person's right to family contact (essential visits are facilitated with precautions, not banned).

26. CLEANING AND DISINFECTION

Requirement	Standard
Posted schedule	Appendix E is the posted cleaning schedule: daily (kitchen surfaces, dining table, bathrooms, high-touch points — door knobs, light switches, handrails, remotes, shared devices), weekly (floors throughout, bedrooms with the person's participation as appropriate, fridge check), and periodic tasks. Completion is initialled.
Products	An approved household disinfectant (or a prepared bleach solution: ~1:50 for body fluid contamination, ~1:500 general disinfection, made fresh daily and labelled) used per label contact time. Cleaning removes soil first; disinfection follows — a disinfectant wiped off immediately has done nothing.
Chemical safety	Products stored locked (facility safety standard), never mixed (bleach + ammonia produces toxic gas), labelled, with gloves used per label; WHMIS/OHS requirements per the OHS Binder.
Enhanced cleaning (illness in the home / outbreak)	During GI or respiratory illness: high-touch surfaces at least twice daily; the ill person's washroom cleaned last with dedicated cloths; disposable cloths preferred; for suspected norovirus, use a high-level bleach protocol per BCCDC guidance.
Shared items	Personal care items (toothbrushes, razors, towels, water bottles) are never shared; toys and shared items (M) washable and cleaned on schedule and after illness.

27. LAUNDRY AND WASTE

- Soiled laundry is handled with minimal agitation, gloves for body-fluid-soiled items, carried in a basket or bag (never against the body), washed warm/hot with detergent, and dried thoroughly; body-fluid-soiled items are washed separately.
- General waste is bagged and removed daily from kitchen and bathrooms; waste containing body fluids is double-bagged.
- Sharps (from any injectable medication, lancets) go directly into the approved sharps container per ECC-MED practice; the container is sealed and exchanged at the pharmacy before full.

28. BLOOD AND BODY FLUID SPILLS AND EXPOSURES

Situation	Response
Spill cleanup	Gloves (plus eye protection/apron for splash risk); contain and absorb with paper towel; clean with detergent; disinfect with bleach solution 1:50 (contact time per label); discard materials double-bagged; hand hygiene.
Staff exposure (needlestick, bite breaking skin, body fluid to eyes/mouth/non-intact skin)	First aid immediately (wash wound with soap and water; flush eyes/mouth with water); report to PC/PM immediately; seek medical assessment the same day (ED for significant exposures — post-exposure prophylaxis is time-sensitive); WorkSafeBC report per the OHS Binder; document.
Person-in-care exposure	Same first aid; clinical advice same day (811/prescriber/ED per severity); consent framework per ECC-HC; notifications per Section 20 if serious.

29. OUTBREAK RECOGNITION AND MANAGEMENT

An outbreak for program purposes: **2 or more persons (residents or staff) with similar GI or respiratory symptoms within 48–96 hours**, or any single case of a serious communicable disease (e.g., measles, pertussis, invasive group A strep, COVID-19 per current public health direction). When in doubt whether it is an outbreak, call public health and ask — that call is free and the delay is not.

Step	Action	Owner / Timeline
1. Recognize and declare internally	PC identifies the case pattern, starts the line list (Appendix F), and notifies the PM the same day.	PC — same day
2. Call public health	PM (or PC) calls the local public health unit / environmental health officer, reports the situation, and follows all direction given; direction received is documented on Appendix F.	PM — same day
3. Implement control measures	Enhanced cleaning (Section 26); exclusion per Appendix G; cohorting of ill persons to their rooms/washroom as layout allows; PPE for close care; suspend	Shift Lead / all staff — immediately

Step	Action	Owner / Timeline
	communal food preparation by ill or exposed persons; increase hand hygiene support.	
4. Maintain the line list	Every symptomatic person (residents and staff): name, onset date/time, symptoms, severity, care sought, exclusion dates, resolution. Updated every shift.	PC — every shift until declared over
5. Notify the funder	(M) Social worker informed; licensing informed where the facility is licensed. (C) CLBC informed per critical incident/contract terms. Families/consent contacts informed appropriately (ECC-HC frameworks).	PM — same day
6. Monitor and escalate individuals	Each ill person is monitored per Part 3; dehydration and respiratory red flags escalate per Sections 15–17 — outbreak management never dilutes individual care.	All staff — continuous
7. Declare over and debrief	The outbreak is declared over per public health direction (commonly after a defined symptom-free period). The PM debriefs within 5 business days: source hypotheses, what worked, gaps, corrective actions with owners.	PM — within 5 business days of end

30. PUBLIC HEALTH NOTIFICATION AND REPORTING

- The local public health unit / health authority communicable disease line is recorded on the program contact sheet and Appendix D; the PM verifies the number annually.
- Suspected outbreaks (Section 29) and any single case of a disease public health has asked to be notified of are reported the same day; the program follows and documents all Medical Health Officer direction, which prevails over this manual where stricter.
- Immunization is the standing outbreak defence: the program maintains current immunization per ECC-HC §9, offers seasonal influenza (and other publicly funded) vaccines annually, and supports staff immunization; during certain outbreaks public health may direct exclusion of non-immunized persons — the program complies and supports those affected.
- Records of any outbreak (line list, direction received, notifications) are retained with program records and available to the funder, licensing, and public health.

PART 5 — Quality Assurance

31. QA INDICATORS

Indicator	Target	Frequency
Current 4-week menu posted; quarterly refresh completed with resident participation	100%	Quarterly PM approval
Appendix B (diet/texture/allergy plan) current for every person and reconciled with ECC-MED §7 and ECC-HC App. A	100%; discrepancies fixed same day	Monthly PC audit; annual review
Fridge/freezer temperature logs complete	100% of days	Monthly facility checklist
FOODSAFE coverage — every food-preparation shift	100%	Monthly staff currency check (by the 5th)
First aid currency — all staff	100%	Monthly staff currency check
Illness/injury reports (Appendix C) completed before end of shift; PM review within 24 hours	100%	Per event; monthly audit
Funder notifications for serious illness/injury made within required timelines	100% — any miss is itself an incident	Per event; PM confirmation
Cleaning schedule (Appendix E) completion	≥ 95% of scheduled tasks initialled; misses explained	Monthly PC audit
Staff exclusion compliance (no staff worked while excludable)	100%	PC tracking; PM review
Outbreaks: public health called same day; line list maintained; debrief within 5 days of end	100%	Per outbreak
EpiPen (where applicable) current, accessible; anaphylaxis-allergen kitchen controls in place	100%	Monthly inspection (with ECC-MED §23.2)

32. ANNUAL REVIEW AND TRAINING

- This manual is reviewed annually by July 1 by the PM and approved by the ED, and immediately upon any public health or funder directive, serious incident, or regulatory change.
- Orientation (before unsupervised duties) covers: the menu and Appendix B system; allergy board and anaphylaxis response; texture plans in the home; the illness/injury response ladder and Section 20 notification table; hand hygiene, exclusion, and cleaning duties. Annual refresher for all staff; competencies confirmed by the PC.
- A mock exercise is run at least annually (and at program opening per ECC-12BED §2.5): an illness/injury tabletop including the notification chain, or an outbreak tabletop including the line list and public health call.
- Training completion is recorded in personnel files (Appendix H) and tracked in the monthly staff currency check.

PART 6 — Appendices

APPENDIX A — 4-WEEK MENU PLAN TEMPLATE

One page per week of the rotation. Post the current week in the kitchen. Note day-of substitutions directly on the posted copy. Retain posted menus 12 months. Individual diet/texture/allergy variations per Appendix B are shown in the Variations row.

Week Number (1-4) / Effective Dates							
Prepared By (PC) / Date							
Resident Input Recorded (how / when)							
PM Approval — Signature / Date							
Meal	Monday	Tuesday	Wednesday	Thursday	Friday	Saturday	Sunday
Breakfast							
AM Snack							
Lunch							
PM Snack							
Dinner							
Evening Snack							
Variations (per Appendix B — by person)							
Substitutions made (date / by / what)							

APPENDIX B — SPECIAL DIET, TEXTURE AND ALLERGY PLAN (PER PERSON)

One plan per person; completed by the PC at admission; approved by the PM; reconciled with the Individual Medication Profile (ECC-MED §7) and Health Care Profile (ECC-HC App. A); reviewed at every change and annually. Summarize allergens on the kitchen allergy board.

Person's Legal Name	
Date of Birth	
Stream (M / C)	
FOOD ALLERGIES — allergen; reaction type; severity; EpiPen prescribed? (location per ECC-MED §23.2)	
Intolerances / sensitivities (non-allergic) and accommodation	
Medical diet(s) ordered — diet;	

prescriber/dietitian; date of order (attach order)	
Texture modification — IDDSI food level; IDDSI fluid level; ordered by; date (attach assessment)	
Mealtime supervision / positioning / pacing / adaptive equipment requirements	
Choking-risk foods to avoid or modify for this person	
Cultural / religious food requirements and observances	
Strong preferences and aversions	
Nutrition goals in the person's plan (if any) and supports	
Reconciled with ECC-MED \$7 profile and ECC-HC App. A (date / initials)	
PC Signature / Date	
PM Approval — Signature / Date	
Annual Review Dates / Initials	

APPENDIX C — ILLNESS / INJURY REPORT AND NOTIFICATION CHECKLIST

Complete before end of shift for every illness or injury beyond minor first aid, every 911 call, and every event in the Section 20 table. Objective language: what was observed, what was done, times. PM reviews within 24 hours.

Person's Legal Name	
Date and Time of Onset / Event	
Date and Time Discovered / Reported	
Type (circle): Illness / Injury / Choking / Anaphylaxis / Seizure / Head injury / Mental health crisis / Other	
Description — what was observed (signs, symptoms, mechanism of injury)	
First aid / immediate care provided — by whom, when, person's response	
Clinical advice sought: 811 / prescriber / Poison Control — time, advice received	
911 called? (time; EMS outcome) / ED or urgent care? (facility; accompanied by)	

Outcome — condition at end of shift; admitted? discharge instructions attached?				
Notification (per Section 20)	Required? (Y/N)	Name Notified	Date / Time	By
PC / on-call supervisor				
Program Manager				
MCFD social worker (M) (after hours 1-800-663-9122)				
Licensing Officer (M — licensed facilities)				
CLBC (C) — verbal + written CI per contract				
Family / consent contacts (per ECC-HC framework)				
Public health (if communicable / outbreak)				
CFCSA s.14 / adult protection report (if applicable)				
Follow-up actions required (appointments per ECC-HC App. E; new prescriptions per ECC-MED §8; restrictions; monitoring)				
Staff Member — signature / date				
PC Signature / Date				
PM Review — signature / date (within 24 hours)				

APPENDIX D — MEDICAL EMERGENCY QUICK REFERENCE (POST BESIDE EVERY PHONE)

CALL 911 IMMEDIATELY FOR	
Unresponsive or hard to rouse · difficulty breathing · choking not relieved · blue lips	Call 911
Anaphylaxis — throat swelling, hives + breathing trouble	EpiPen FIRST, then 911 (ECC-MED §23)
Suspected overdose or poisoning	911 + Poison Control 1-800-567-8911
Seizure over 5 minutes / first seizure / repeated seizures	Call 911 — do not restrain; time it
Chest pain · stroke signs (FAST) · major bleeding · serious burns · suspected spinal injury	Call 911 — keep the person still

CALL 911 IMMEDIATELY FOR	
Suicide attempt / serious self-harm	Call 911 — stay with the person
Other Numbers	
HealthLink BC — 24/7 nursing advice	811
BC Poison Control (24/7)	1-800-567-8911
On-Call Supervisor	[posted weekly schedule]
Program Prescriber / Clinic (after hours)	[insert]
MCFD Centralized Screening (after hours) (M)	1-800-663-9122
CLBC contact per contract (C)	[insert]
Local Public Health Unit / communicable disease line	[insert — PM verifies annually]
Licensing Officer (licensed facilities)	[insert]

APPENDIX E — CLEANING AND DISINFECTION SCHEDULE

Post in the utility area. Initial each task when done. During illness in the home or an outbreak: high-touch surfaces at least twice daily (Section 26).

Frequency	Task	Mon	Tue	Wed	Thu	Fri	Sat	Sun
Daily	Kitchen counters, sink, dining table (before/after meals)							
Daily	Bathrooms: toilet, sink, taps, high-touch							
Daily	High-touch points: door knobs, switches, handrails, remotes, shared devices							
Daily	Kitchen floor sweep; garbage out (kitchen/bath)							
Daily	Fridge/freezer temperature logged (4°C / -18°C)							
Weekly	Floors throughout; vacuum; mop							
Weekly	Bedrooms (with the person, per their plan); linen change							
Weekly	Fridge contents check — expired/leftovers out							
Monthly	Deep clean rotation (oven, fridge interior, vents, walls spot); first aid kit check; PPE stock check							
Week Of / Completed By								

Frequency	Task	Mon	Tue	Wed	Thu	Fri	Sat	Sun
PC Monthly Audit — signature / date								

APPENDIX F — OUTBREAK LINE LIST AND MANAGEMENT CHECKLIST

Start at 2+ linked GI or respiratory cases (Section 29). Update every shift. This is the record public health will ask for.

Outbreak Type (GI / respiratory / other)					
Date Recognized / By					
Public Health Called — date, time, person spoken to					
Direction Received From Public Health (attach notes)					
Name (resident/staff)	Onset Date/Time	Symptoms	Care Sought / Outcome	Excluded From / Dates	Resolved (date)
Control Measure Checklist				Started (date/initials)	
Enhanced cleaning — high-touch 2x+ daily; ill person's washroom last / dedicated					
Exclusions applied per Appendix G (residents and staff)					
PPE in use for close care; hand hygiene supplies checked					
Communal food prep by ill/exposed persons suspended					
Funder notified: (M) social worker + licensing (licensed) / (C) CLBC					
Families / consent contacts informed (ECC-HC framework)					
Declared over (per public health) — date					
PM debrief within 5 business days — date; corrective actions logged					

APPENDIX G — EXCLUSION (STAY-HOME) QUICK REFERENCE

Symptom / Condition	Person in Care	Staff Member
Vomiting or diarrhea	No school/work/day program/food prep until 48 hours symptom-free	NO WORK until 48 hours symptom-free — no exceptions
Fever (38°C+) / influenza-like illness	Home until fever-free 24 hours without fever medicine and improving	No work until fever-free 24 hours and improving
Strep throat	Until 24 hours of antibiotics completed and well enough	Until 24 hours of antibiotics and well enough
Conjunctivitis (pink eye)	Per prescriber direction; strict hand hygiene	Per prescriber; no close care until treated/cleared
Chickenpox / shingles (open lesions)	Per public health / prescriber (chickenpox: until lesions crusted)	Per public health / prescriber
Infected skin lesions on hands	Covered; no food prep participation until healed	Fully covered + no food prep; if not coverable — no work
COVID-19 / other diseases with current public health direction	Per current public health guidance	Per current public health guidance
Undiagnosed rash with fever	Home + same-day clinical advice (811/prescriber)	No work until assessed

When unsure: call 811 or the public health unit. The PC decides borderline resident exclusions; the PM decides borderline staff exclusions. Exclusion is documented (shift log / Appendix C; staff exclusions in the staffing record).

APPENDIX H — STAFF ACKNOWLEDGEMENT

All staff must read and sign this acknowledgement before unsupervised duties and upon each annual update of this manual.

I acknowledge that I have received, read, and understood the Evergreen Community Care Nutrition, Illness & Infection Prevention Policy Manual (ECC-NIP-2026-001), Version 1.0, effective July 1, 2026.	Initials
I check the kitchen allergy board and each person's Appendix B plan before preparing or serving any food, and I never change a texture level without a clinical order.	
I understand food is never withheld, delayed, or used as a reward or consequence.	
I know the anaphylaxis response: EpiPen immediately, 911 immediately (ECC-MED §23).	
I monitor wellness every shift, report concerning changes the same shift, and use the escalation ladder: 811 / prescriber → urgent care → 911. I will never be criticized for calling 911 in good faith.	
I know the Section 20 notification table: serious illness or injury means the social worker and licensing where applicable (M) or CLBC (C) are notified within required timelines, and I start the chain by notifying the PC immediately.	
I practice hand hygiene at the essential moments and complete my assigned cleaning tasks per	

I acknowledge that I have received, read, and understood the Evergreen Community Care Nutrition, Illness & Infection Prevention Policy Manual (ECC-NIP-2026-001), Version 1.0, effective July 1, 2026.		Initials
the posted schedule.		
I do not work with vomiting or diarrhea until 48 hours symptom-free, or with fever until fever-free 24 hours — regardless of staffing pressure.		
I know that 2 or more linked GI or respiratory cases triggers the outbreak protocol and a same-day call to public health.		
I complete the Illness/Injury Report (Appendix C) before the end of my shift for any reportable event.		
Staff Name (print)		
Position		
Program		
Signature		
Date		
Current First Aid Expiry		
FOODSAFE (if held) Expiry		
PC Signature (training current, on file) / Date		

VERSION HISTORY

Version	Date	Summary
1.0	July 1, 2026	Initial Nutrition, Illness & Infection Prevention Policy Manual: nutrition and menu policy per Canada's Food Guide (2019) with 4-week rotation, medical/cultural diets, IDDSI texture modification, food allergy and anaphylaxis management (kitchen allergy board reconciled with ECC-MED \$7 and ECC-HC App. A), choking prevention and FOODSAFE food safety standards; illness, injury and medical emergency policy with escalation ladder (811 → urgent care → 911), head injury/seizure/diabetic/sepsis protocols, and the Section 20 notification matrix (M: MCFD social worker + licensing where licensed; C: CLBC critical incident reporting); infection prevention and outbreak policy with routine practices, hand hygiene, PPE, exclusion standards for persons in care and staff, cleaning/disinfection schedule, blood and body fluid exposure response, outbreak protocol (2+ linked cases → same-day public health call, line list, debrief); QA indicators; 8 appendices.

Executive Director	Signature: _____ _____	Date: _____
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